



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هاله محمد صلاح الحسيني

العنوان : 0

الحي / المدينة : الهرم

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 26304201301987 رقم التليفون : 01092980736

العمر (تاريخ الميلاد) : 20-04-1963 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: هاله محمد صلاح الحسيني National ID: 26304201301987	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

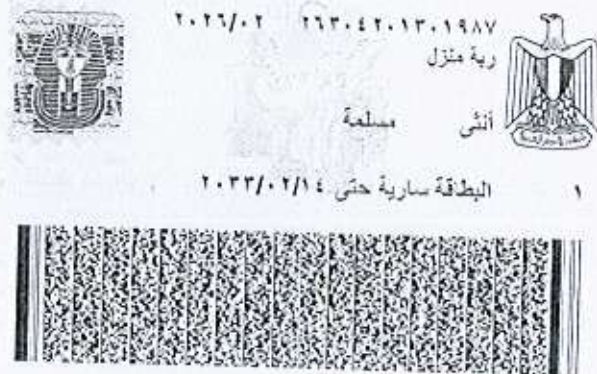
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



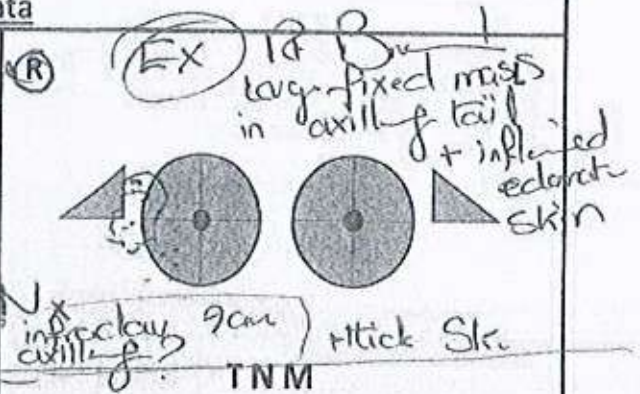
MDT FORM

9 / 7 / 20 26 G

2234
26

Name	دالة عرصة	MRN	116 98
Age	63	ID	263 04201301987
Menopausal status	Post Menopausal	Phone	01092980736
NRS SCORE	0	Marital status	
BMI		Family history	(+) Return + Breast + CK at Sites
CO morbidity	DM + HTN	REASON OF MDT	New Case

Clinical Data			
BX:	R-Breast + axillary lymph nodes		
ER:	PR:	HER 2:	Ki67:
0/8	0/8	0+	50%
Sonomammography: 4/6/2026			
huge mass 9 → 2 o'clock zone C axillary tail 22 x 11 cm +			
Mets Workup:			
Bone Scan 2/7/2026			
equivalant uptake Lt. lower			
CT-CAP 1/7/2026			
Recommendations:			



for Echo, Porta Cath & Labs:
Start Metastatic chemo accordingly to Keynote 522 Protocol
Surgical Option accordingly to response + BRCA test "very strong family history"

MDT OUTCOME & PLAN
To start NACT: Taxo-Carbo-Keytruda x 4 weeks
then AC-Keytruda x 4
then MRN guided by response

+ BRCA test					
SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	هالة محمد صلاح	Case No.	317554
Sex/Age	Female 63 Y	Patient No.	23929
Registered Date	20/6/2026 03:49:44 PM	Referred By Dr.	Out Patient
Validation Date	07/7/2026 11:04:17 AM	Branch	Gustave Roussy
Reporting Date	07/7/2026 11:41:24 AM	Pathology No.	SR-714/26

Breast Slide Revision

Clinical Data: Right breast UOQ mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (1063516). Together with, five referred slides including 1 (H&E) stained slide and 4 immunostained slides for (ER, PR, Her2 & Ki67).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma with medullary like features.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angio-invasion: Absent.
- * Peri-neural invasion: Absent.

IPT:

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).
- Ki67%: 50%.

Diagnosis:

- Right breast mass, Referred block & slides (Tru-cut biopsy):**
- Invasive duct carcinoma with medullary like features, Grade 3.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Patient Name	فاطمة محمد صلاح	Case No.	317554
Sex/Age	Female 63 Y	Patient No.	23929
Registered Date	20/6/2026 03:49:44 PM	Referred By Dr.	Out Patient
Validation Date	07/7/2026 11:04:17 AM	Branch	Gustave Roussy
Reporting Date	07/7/2026 11:41:24 AM	Pathology No.	SR-714/26

IPT:

- ER: Negative (0/8).
- PR: Negative (0/8).
- Her2: Ultra-low (score 0+).
- Ki67%: 50%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

****Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature**Prof. Dr. Maha Guimei.**

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt



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Patient's Name: Hala Mohammed Salah El-Husseini
Date: 2026-06-04
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY
REVEALED:**

INDICATION:

Right axillary tail huge palpable mass

FINDINGS:

right breast :

the entire breast shows thickened skin associated with increased parenchymal density compared to the left side

by complementary ultrasound:

- thickened trabeculae and skin reaching to 18 mm
- huge heterogenous mass extending from 9 o'clock to 2 o'clock zone C and axillary tail , With cystic degeneration as well as markedly increased vascularity within , measuring roughly 22 x11 cm
- axillary lymph nodes cannot be assessed adequately, occupied by the huge mass
- multiple enlarged infraclavicular lymph nodes , the largest measures 9.6 x 4.2 mm

left breast :

- Scattered fibroglandular tissue ACR B.
- no speculations , masses , assymetry or pathological microcalcifications
- Complementary U/S no suspicious solid or cystic lesions
- few enlarged benign looking axillary lymph nodes with preserved oval shape and central echogenic hilum

CONCLUSION:

- right highly suspicious mass for histopathological correlation (BIRADS 5)
- Left normal study (BIRADS.1)

Dr farah elshobaky

ECHO REPORT

NAME: علاء محمد DATE: 13/7/2026
ID NUMBER: ٢٢٢٤ / ٢٢٦ SEX: ♀ AGE: 63

EF	66%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.7	3.5-5.8cm	AO.R	2.9	3-3.7cm
LVESD		2-4cm	LA	4.0	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions incl

LV Wall Thickness incl

LV Diastolic Function DD grade I

SWMA SWT 1A

LA and Ao Root Dimensions incl

Mitral Valve ild MR

Aortic Valve Sclerotic A-V with no sig. systolic gradient

Tricuspid Valve ild TR

Pulmonary Valve incl

Right Side and Systolic Function incl

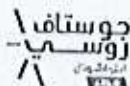
Intracardiac Masses and Effusion incl

DR.

Ch. Lin/3

Patient's Name: Hala

Mohammed Salah



Date: 2026-07-01

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt

MULTISCLICE C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER I.V. CONTRAST INJECTION REVEALED:

Known case of breast CA. Right breast is partially out of field.

I- CHEST:

- Large right breast lesion is seen measuring 20 x 12 cm associated with heterogenous breast parenchyma and skin thickening for sono-mammography.
- No pulmonary nodules, masses, consolidation, collapse or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Mildly enlarged liver showing homogenous parenchymal attenuation. No focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness, nephrogram and excretory function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- No definite lytic or sclerotic lesion

OPINION:

- ↓ No pulmonary , hepatic or osseous metastatic lesions.

Dr.sherif Wagdy

Name : Hala Mohammed Salah Al-Husseini
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 02-07-2026 09:50
Medical No. : 407675

WHOLE-BODY BONE SCINTIGRAPHY REPORT

Clinical Indication: Initial staging and follow-up evaluation of known right-sided breast carcinoma.

Technique: Anterior and posterior whole-body planar scans acquired following IV injection of Tc-99m MDP.

FINDINGS

- Relatively uniform and symmetric radiotracer distribution throughout the skull, spine, pelvis, rib cage, and extremities. A small, focal area of equivocal increased uptake is noted at the left femoral head.
- No other suspicious focal or widespread osteoblastic activity identified.
- Prominent large soft-tissue silhouette projecting from the right chest wall and breast region with faint, diffuse tracer localization, correlating with the known primary right breast tumor.
- Normal physiological tracer localization in both kidneys and urinary bladder. No obstruction.

IMPRESSION

- Equivocal focal uptake at the left femoral head warrants dedicated radiological correlation (X-ray, CT, or MRI) to differentiate between early metastatic disease and benign degenerative changes.
- No definite scintigraphic evidence of multifocal or widespread bone metastases.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Patient's Name:

Hala Mohamed Salah

Date:

2026-09-01

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MRI EXAMINATION OF THE HIP JOINTS:

MR Technique:

- Axial T1, T2 & T1 FS, T2 FS
- Coronal T1 & T2 FS
- Sagittal T1 & T2 WI

MR Findings:

- Bilateral mild effusion of both hip joints.
- Maintained normal articular relation of both hip joints.
- Preserved normal spherical configuration of both femoral heads.
- Normal marrow signal of the examined bones.
- Normal MR signal of the peritarticular muscles.
- No peritarticular masses, collection or bursitis.
- Intact both sacro-iliac joints.
- A tiny cyst is seen in the adnexa,inseperable from the ovaries and being larger on the left side and measures about 23mm.
- No sizable pelvic lymphadenopathy.
- Mild free fluid is seen in the Douglas's pouch.

IMPRESSIONS:

- Bilateral mild effusion of both hip joints.
- A tiny cyst in the adnexa,inseperable from the ovaries and being larger on the left side.
- Mild free fluid in the Douglas's pouch.
- Otherwise,normal study.

Prof/Hanan El-Ahwal.

Patient's Name:

Hala Mohamed Salah

Date:

2026-09-01

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

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MR Technique:

- Axial T1, T2 & T1 FS, T2 FS
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IMPRESSIONS:

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- A tiny cyst in the adnexa,inseperable from the ovaries and being larger on the left side.
- Mild free fluid in the Douglas's pouch.
- Otherwise,normal study.

Prof/Hanan El-Ahwal.

اسم الفرع فرع جناح , الازهار 3
مدير الفرع دكتور بدر محمود هلال
مدير الوحدة ا.د رانيا دارويش
طبيب الوحدة ا.د. عطية الش

رئيس مجلس الادارة
ا.د. موفيه كامل
مدير المعامل
ا.د. هناد الشربيني

PATIENT NAME هالة محمد صلاح الحسني			Registered 28-08-2026 19:17:49	Collected 28-08-2026 19:18:00
Visit Number 75426507478	Age 63 Year	Gender Female	Authenticated 18-09-2026 09:46:53	Printed 18-09-2026 21:02:06
Referred By Prof : -			Client ID 55452	

MOLECULAR BIOLOGY REPORT

BRCA 1,2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result: A heterozygous variant of uncertain significance was detected in *BRCA1* gene by sequencing.

NM_007294.4(*BRCA1*): NM_007294.4: c.4211T>C (p.Leu1404Pro) (rs80356916)

Variant interpretation:

-The p.L1404P variant located in coding exon 11 of the *BRCA1* gene, results from a T to C substitution at nucleotide position 4211. The leucine at codon 1404 is replaced by proline, an amino acid with similar properties. This amino acid position is highly conserved in available vertebrate species. In addition, this alteration is predicted to be deleterious by in silico analysis. Based on the available evidence, the clinical significance of this variant remains unclear.

-This variant has extremely low frequency in gnomAD population databases

-The variant is classified as variant of uncertain significance by ACMG guidelines (PM2,PP3,PM1).

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the *BRCA1* & *BRCA2* genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.

- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.

- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, *BRCA2* carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay
- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of *BRCA1* & *BRCA2* genes.
- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

اسم القرع فرع حانة, الاعداد 3
مدير القرع د.سحر بلر محمود هلال
مدير الوحدة ا.د رانيا درويش
طبيب الوحدة ا. د. علياء الشش

رئيس مجلس الإدارة
ا. د. مؤمنة كامل
مدير المعامل
ا. د. هندا الشربيني

PATIENT NAME هالة محمد صلاح الحسيني			Registered 28-08-2026 19:17:49	Collected 28-08-2026 19:18:00
Visit Number 75426507478	Age 63 Year	Gender Female	Authenticated 18-09-2026 09:46:53	Printed 18-09-2026 21:02:06
			Referred By Prof : -	Client ID 55452

(Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.

- Benign and likely benign variants are not included in this report.

- Variants of unknown significance classification may change over time.

Limitations

-Sanger sequencing of the detected variant is recommended.

- Large rearrangements may not be detected; variants in regions with high GC content or pseudogenes may give false negative or false positive results.

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

Recommendations

- Genetic counseling may be of benefit.

- At-risk family members should be offered testing for the identified pathogenic variant.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.

- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

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